

إسم المريض رابعياً : محمود عبدالظاهر زكى عبدالله

المحافظة : الجيزة **الحى / المدينة : العمرانية**

العنوان : العمرانيه

رقم البطاقة : 27902192301098 رقم التليفون : 01000293861

تاريخ تسجيل الإستمارة : 23-09-2026

العمر(تاريخ الميلاد) : 19-02-1979 المهنة : عامل عادي

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : اورام ليفاويه

نتائج الأبحاث : (مرفق)

التشخيص النهائي : اورام ليفاويه

خطة العلاج : أورام كيماوي ابحاث وعينه [10853] **قسم العلاج :** الاورام

التعليق على التشخيص : -----

GR 1-4
ر. د. يوسف
أستاذ
الدراسات
الاسلامية
جامعة
البحرين
صحة البيانات المرفقة
الاجتهاد

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نَفَقَات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: محمود عبدالظاهر زكي عبدالله National ID: 27902192301098	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of lymphoma for Clinical research and specimen
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

محمود
عبد القادر زكي مصطفى
مستشار ومساعد مدير المباحث الجنائية
الهيئة الوطنية للأمن القومي

٢٠٢٣/٠١ ٢٧٩.٢١٩٢٣.٠١-٩٨

JV3482223

٢٠٢٣/٠١ ٢٧٩.٢١٩٢٣.٠١-٩٨

ذكر مسلم متزوج

البطاقة سارية حتى ٢٠٢٣/٠١/١٥

١٢٩٤٨

Patient Name: MAHMOUD ABDELZAHER ZAKI ABDALLAH AGE: 47 Y 03 M 13 D
MRN: 10110225014314 Location: البجدة
D.O.B: 19/02/1979 Sex: أمراض
IC No.: 27902192301098 Male

HEMATOLOGY

BONE MARROW TREPHINE BIOPSY

Specimen Id 2605160510
Accession No Hem-26-196952
Sample Type Bone marrow
Collection Date 16/05/2026 11:27 AM
Received Date 16/05/2026 12:48 PM
Doctor أمراض الدم

Specimen number: 521
PB Findings : normal TLC, no anemia, normal platelets.
BMA Findings : hypercellular for age, increased megakaryocytes, 2% blasts, 5% eosinophils, 3% plasma cells
BMB Findings:
Site: PSIS, unilateral
Fixation and Decalcification method:
BMB Gross examination: BM trephine biopsy core(s): 1cm, 1.5cm, fragmented bony core
Histological examination of BM trephine biopsy sections revealed:
Age-related overall marrow cellularity & fat spaces: HYPOCELLULAR FOR AGE, 40% cellularity, adequate >5 intratrabecular areas.
Hematopoiesis:
Erythroid: normal in count, morphology, maturation and distribution.
Granulocytic: normal in count, morphology, maturation and distribution.
Megakaryocytic: normal in count, morphology, maturation and distribution.
Abnormal infiltrations: mild interstitial increase in plasma cells, increase eosinophils.
Reticulin fibers and Collagen bundles contents & distribution: MF-1: Loose network of reticulin with many intersections, especially in perivascular areas
BM trabeculae: Normal
Others: D138(10-15%)
Immunohistochemistry: Was performed on formalin-fixed, paraffin-embedded tissue sections using the Dako EnVision™ FLEX detection system
Conclusion: Known case of MM, hypocellular for age showing mild interstitial increase in plasma cells confirmed by IHC.
Further recommended investigation:
Signature:



Exam Information:

Patient Name: MAHMOUD ABDELZAHER ZAKI	Date of birth: 19/02/1979
Arabic Name: محمود عبد الظاهر زكي	Patient Gender: M
Patient RIS ID: 384633	Exam Date: 24/05/2026
Patient HIS ID: 10110225014314	
Written by: Dr. Hoda Fathy	Approved by: Dr. Hoda Fathy

Whole body PET/CT EXAMINATION (11-5-2026)

Clinical History:

- Male patient diagnosed with managed plasma cell infiltrate? MM.
- PET/CT requested for re-assessment.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROIs and the normal threshold is < 2.5 .

Report:

Compared to previous PET/CT report dated 5-7-2025:

Hepatic Reference SUVmax~2.3.

Head and Neck:

- * Currently detected Rt cerebral parieto-temporal hypodensity for clinical and local MRI data correlation.
- * Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx, showing normal appearance.
- * Physiologic FDG uptake is seen in submandibular and parotid salivary glands, showing normal appearance.
- * Otherwise, Normal size, enhancement & symmetrical appearance of thyroid glands with no FDG avid lesions detected.
- * Normal appearance of the neck muscles & vessels.
- * No hypermetabolic cervical lymphadenopathy detected.
- * No FDG avid neck soft tissue mass lesions.

Chest:



- * Still noted old bilateral gynecomastia, the Rt side is still bulkier with no significant FDG activity (was of low grade activity of SUVmax~1.6), for clinical and/ or sono-mammographic correlation.
- * Still noted old bilateral axillary LNs with no significant FDG uptake and preserved fatty hila (was of low grade activity), likely non specific.
- * No hypermetabolic mediastinal lymphadenopathy.
No FDG avid lung nodules or masses detected apart from still noted bilateral fine apical reticulations, yet previous few small bilateral sub-pleural nodules are not appreciated on current scan.
- * No metabolically active pericardial or pleural lesions detected.
- * Physiologic FDG uptake is noted in the heart, showing normal shape.

Abdomen and Pelvis:

- * Physiologic diffuse FDG uptake is seen over still enlarged liver with homogenous parenchymatous texture. No FDG avid focal lesions detected *apart from still noted few small hypodense non FDG avid hepatic cysts, for US correlation.* Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries. No intra-hepatic or extra-hepatic biliary radicle dilatation.
- * Physiologic diffuse FDG uptake is seen over still enlarged spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- * Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- * Normal appearance of the pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.
- * No hypermetabolic abdominal or pelvic lymphadenopathy.
- * No FDG avid ascites or abdominal collections noted.

Musculoskeletal:

- * Moderate- remarkable metabolic regression of old reported FDG avid large Lt pelvic bone (posterior acetabulum column and ischium) lytic osseous lesion eliciting SUVmax~2.2 (was~15.4) as well as other few osseous lesions seen at DV2 Rt lamina of SUVmax~2.1 (was~4.9) and S2 with no significant FDG uptake (was of SUVmax~5), for FU.



- Currently depicted low grade FDG avid DV6 sclerotic focus of SUVmax~2.6, for close FU.

Conclusion:

Follow up PET-CT study shows:

Metabolic regression of old few FDG avid lytic osseous (myelomatous) lesions.

Short term FU PETCT may be advised.

Thank you for referral

Dr. Nada Fadl, MD

Nuclear Medicine



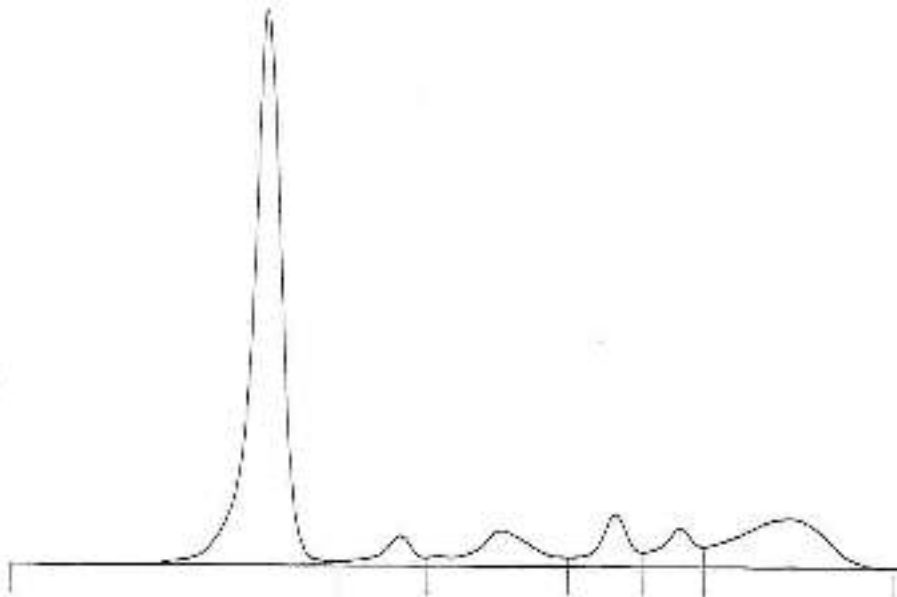
Clinical and Chemical Pathology Department
Clinical Chemistry Unit

ID: 10110225014314

Mahmoud Abdel Zaher Zaky

Sample #:3

Date:14/5/2026



Serum protein electrophoresis

Fractions	%	Ref. %	Conc.	Ref. Conc.
Albumin	60.6	55.8 - 56.1	4.1	3.5 - 5.0
Alpha 1	3.8	2.9 - 4.9	0.3	0.1 - 0.3
Alpha 2	8.2	7.1 - 11.8	0.6	0.6 - 1.0
Beta 1	6.0	4.7 - 7.2	0.4	0.3 - 0.5
Beta 2	5.1	3.2 - 6.5	0.3	0.2 - 0.5
Gamma	16.3	11.1 - 18.8	1.1	0.7 - 1.6

A/G Ratio:1.54

SPEP shows:

- Normal SPEP pattern.
- No M-band could be detected.

T.P. (6.6-8.3g/dL): 6.8

S. Ca (8.8-10.6 mg/dl): 9.4

S. creat (0.5-1 mg/dl): 0.9

B2M (0.7 - 3.4 mg/L):

Head of unit: Prof. Dr. Imen Attia



Clinical and Chemical Pathology Department

Clinical Chemistry Unit

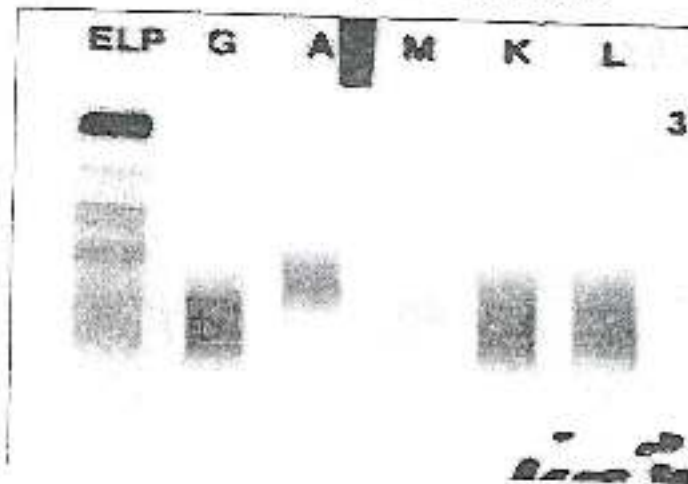
Mahmoud Abdel Zaher Zaky

ID: 10110225014314

Sample #: 11

Date: 21/05/2026

Immunofixation / Bence Jones



Comment: Serum IFX shows:

- Normal immunoglobulin pattern with preserved K/L ratio.
- No M-band could be detected.

Head of unit: Professor Iman Attia Abdelgawad

GUSTAVE
ROUSSY

مستشفى
الجوهرية

ECHO REPORT

NAME: / 06/ 15 30 /
ID NUMBER: /
DATE: 14 / 7 / 2026
SEX: /
AGE: /

EF	66%	55-75%	PWT	0.7	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVESD	2.3	2-4cm	LA	3.3	2-4cm
FS	30%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function Normal
SWMA Normal
LA and Ao Root Dimensions Normal

Mitral Valve
Aortic Valve
Tricuspid Valve
Pulmonary Valve

Right Side and Systolic Function
Intracardiac Masses and Effusion

DR. / 31 / 2.5

NAME: Mahmoud abdelzaher
DATE: 14/6/2026

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (15cm at Rt MCL) homogenous parenchymal echo pattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Prostate: not enlarged, homogenous

Opinion:

Unremarkable study

Best regards
Dr. NERMEEN RAMADAN FEBR,MD

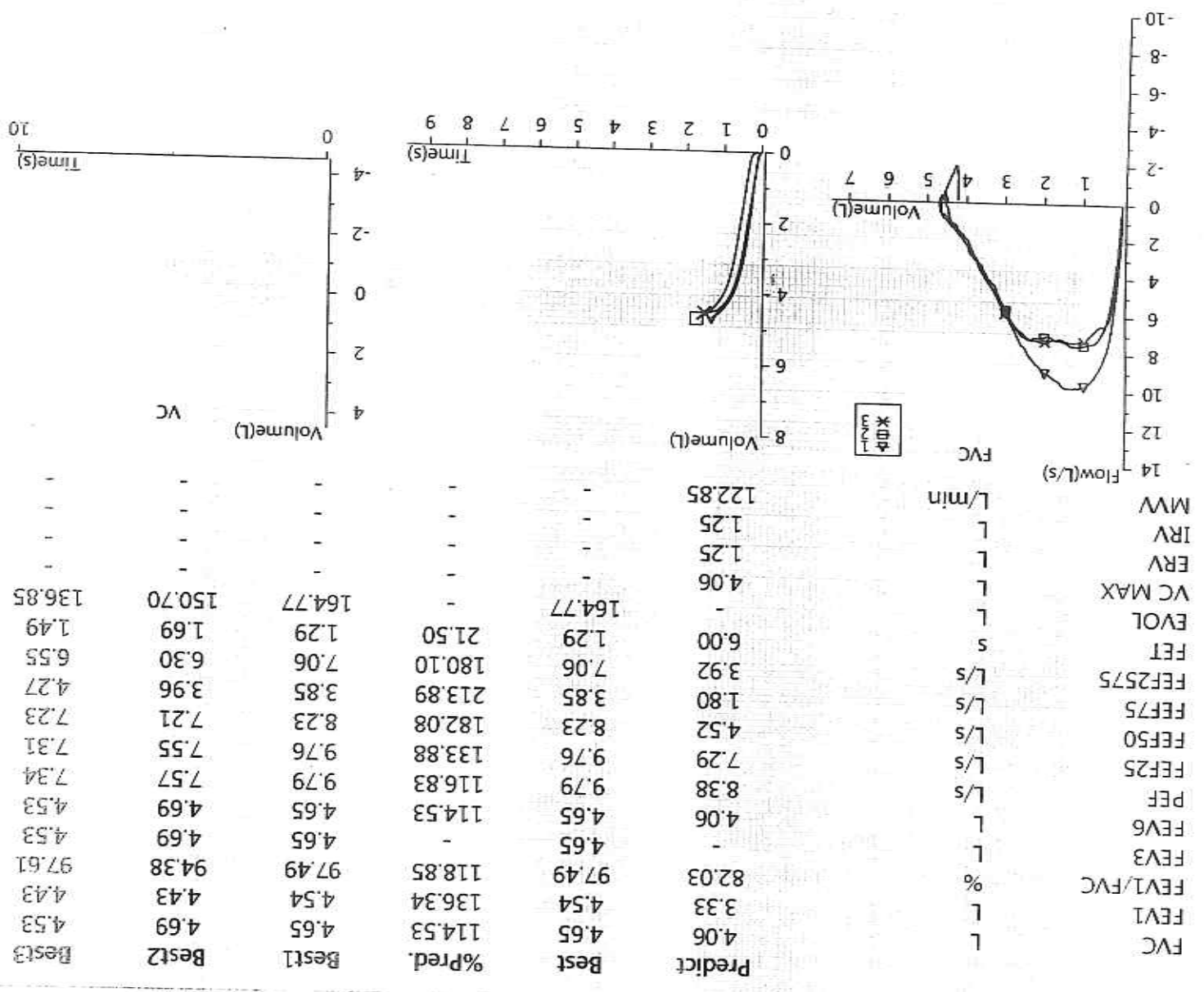


Name: Mahmoud A Alzhr
Height: 167 cm
Department:

Sex: Male
Weight: 70 kg
Patient ID:

Age: 47 Years
Smoke: Never
Predict: ECCS

Handwritten signature and notes in Arabic.



PATIENT'S NAME: محمود عبد الظاهر زكي

AGE: 49Y

SEX: M

REFERRED BY PROF. DR.: Nada Mohsen

CLINICAL DIAGNOSIS: Left iliac bone lytic lesion.

NATURE OF THE SPECIMEN: Core biopsy & aspiration cytology.

SPECIMEN OBTAINED AT: 21/7/2025

SPECIMEN REPORTED AT: 28/7/2025

PATHOLOGY REPORT

GROSS: Two specimens were received: 1- Tiny brownish cores 0.5x0.5cm. totally submitted. 2- 5cc of brown turbid brown fluid for cytological examination.

MICROSCOPY: Sections examined revealed fragments of cellular growth composed of proliferated monomorphic plasmacytoid cells arranged in loose sheets with rare binucleated forms separated by highly vascular fibrous stroma with areas of hemorrhage and hemosiderin granules. Cytological examination revealed markedly hypocellular smears showed only blood component with altered changes.

DIAGNOSIS: LEFT ILIAC BONE OSTEOLYTIC LESION. CORE BIOPSY & ASPIRATION CYTOLOGY.

- FEATURES OF CELLULAR MONOMORPHIC PLASMA CELL INFILTRATE KEEPING WITH PLASMACYTOMA FOR MANDATORY IMMUNOSTAINS AND TIGHT CORRELATION WITH CLINICAL, LABORATORY AND RADIOLOGIC FINDINGS.
- SMEARS OF ALTERED BLOOD COMPONENT.

NB: Three slides enclosed

Thank for reference

SIGNATURE

DR. AMAL HAREEDY, MD

Reported and electronically endorsed by
Dr. Nada Saber

Dr. Mohamed Yousri, MD, PhD

Signed by
Hemorrhagic (non - diagnostic) material. Please follow with the core biopsy result.
Left iliac bone lesion, Fluid Cytology:

Diagnosis
Smears acellular.
Microscopic Description
Two smears, one cell block with one slide were received labeled with patient's name in Arabic.
Natural of Specimen
Slide revision of left iliac bone lesion, fluid collection.
Clinical Picture

Specimen Id	2508191318
Accession No	CYT0-25-1863
Sample Type	TISSUE
Collection Date	19/08/2025 12:25 PM
Received Date	19/08/2025 12:51 PM
Doctor	أبراهيم السيد

CYTOLGY
Slide Revision Cytology

Patient Name:	MAHMOUD ABDELZAHER ZAKI ABDALLAH
MRN:	10110225014314
D.O.B:	19/02/1979
IC No.:	27902192301098
Location:	
Sex:	Male
الهيم	أبراهيم السيد
عينة	



الوزارة
الأبوية
الأبوية
جامعة القاهرة



IC No.:	27902192301098
D.O.B:	19/02/1979
MRN:	10110225014314
Patient Name:	محمود عبدالعزى عتيق
Location:	الم
Sex:	Male

Surgical Pathology-Provisional Diagnosis
Slide/Block revision Pathology

Specimen Id	2508311467
Accession No	P-25-10673
Sample Type	TISSUE
Collection Date	31/08/2025 01:21 PM
Received Date	01/09/2025 08:11 AM
Doctor	المراجى الم

CLINICAL HISTORY

- A new case of 46 years old male patient presented with left iliac bony lesion. Review of cytology material (CYTO-25-1863) revealed hemorrhagic non-diagnostic material.
- Laboratory information done at NCI: BMA (07/08/2025) Normal with 4% plasma cells, BMB (07/08/2025) normocellular with mild interstitial increase in plasma cells highlighted by CD138 (10%). Protein electrophoresis: Increased Alpha2 with polyclonal gammopathy, Beta are normal and No M band detected. Immunofixation revealed IgA, Lambda (double band), B2 microglobulin assay is normal. Serum calcium is low. No evidence of renal insufficiency.
- No radiology available.

GROSS DESCRIPTION

One referred Hx & E stained slide labeled by patient's name.

MICROSCOPIC DESCRIPTION

Section examined revealed extensively hemorrhagic material entangling few foci of malignant tumor formed of sheets of medium sized rounded to plasma cystoid epithelial cells with hyperchromatic nuclei and eosinophilic cytoplasm. These cells show moderate anaplasia and active mitosis.

DIAGNOSIS

Left iliac bone lesion, Referred slide (of biopsy): Hemorrhagic material with focal atypical round cell infiltrate with plasmacytoid features. Block is needed for marker study.
Radiology and full clinical and laboratory findings are needed for proper assessment.

Category

Radiologic data are needed

Assistant Pathologist:
Dr. Samar Ahmad

Registrar:

Signed by:

Dr. Emman Maguib, MD (Assistant Professor)

Reported and electronically endorsed by: Samar Ahmad



IC No.:	27902192301098
D.O.B:	19/02/1979
MRN:	10110225014314
Patient Name:	محمد عبدالعزى جلاله
AGE:	46 Y 07 M 13 D
Location:	الدم
Sex:	Male

HEMATOLOGY
BONE MARROW ASPIRATION

Specimen Id	2508100904
Accession No	Hem-25-215691
Sample Type	Bone marrow
Collection Date	10/08/2025 10:36 AM
Received Date	10/08/2025 01:55 PM
Doctor	المرضى الى

PB Finding :

TLC : Normal
RBCs : No anemia
Platelets : Normal

BONE MARROW ASPIRATION

Site

Right Posterior superior Iliac Spine
High normocellular with many large marrow aggregates
Normal erythropoiesis
Normal granulopoiesis
Normal megakaryopoiesis
with dysmegakaryopoiesis (defective budding and unripening)
and marrow platelet aggregates

Normoblasts :

28

Blasts :

1

Promyelocytes :

2

Myelocytes :

4

Juvenile :

0

Staff :

7

Segmented :

20

Eosinophils :

7

Basophils :

0

Lymphocytes :

16

Monocytes :

2

Plasma Cells :

4 % (with rare binucleated forms)

Promonocytes :

Others :

Cytochemistry :

Conclusion :

A new case referred as suspected plasmacytoma showing high normocellular BM for age
Further Recommended investigations: BMB and myeloma work up investigations.

Sample lab no.

2297

Signature:

Tested by	Dina Siraj
Approved by	Dina Emad El Din Ezzat Ali

Laboratory Report continued , Record No.:10110225014314, Patient Name :MAHMOUD ABDELZAHER ZAKI ABDALLAH.



Patient Name: MAHMOUD ABDELZAHER ZAKI ABDALLAH
MRN: 10110225014314
D.O.B: 19/02/1979
IC No.: 27902192301098

Conclusion:

A case referred as plasmacytoma, BM showing normocellular BM for age with mild interstitial increase in plasma cells highlighted by CD138 (10%).
For correlation with protein electrophoresis and immunofixation.

Further recommended investigation:

Signature:

Tested by	Muhammad Mustafa Ahmed Fouad
Approved by	Hend Abdel Hadi Noah Mahmoud

Procedures	Results	Unit	Reference Range
CA 15-3	9.0	U/mL	(0 - 25)

TUMOR MARKER

8/9/2025

Order by :	إبراهيم الدم
IC No :	27902192301098
Patient Age :	46 Y 05 M 19 D
MRN :	10110225014314
Patient Name:	محمد عبد الحليم
Collect Date:	07/08/2025 12:48
Validation :	09/08/2025 11:11
Location :	عيادة سرطان الدم
Gender :	Male
Sample :	




 جمهورية مصر العربية
 بطاقة تحديد الشخصية
 محمود
 عبدالظاهر زكي عبدالله
 ١١ ش محمد جمال- عبدة بن الجراح
 العمرانية - الجيزة
 ١٩٧٩/٠١/١٩
 ٢٧٩٠٢١٩٢٣٠١٠٩٨
 JV3482223


 ٢٠٢٣/٠١/١٩ ٢٧٩٠٢١٩٢٣٠١٠٩٨
 عامل
 متزوج مسلم ذكر
 البطاقة سارية حتى ٢٠٣٠/٠١/١٥
